YG MAJOR INCIDENT ACTION CARD:
Resident On-Call Anaesthetist (Bleep 063)

TASK	DESCRIPTION	✓
1.	Contact Consultant On-call and inform them of the incident.	
2.	If you are committed in theatres and unable to attend ED, inform the other on-call residents and the on-call Consultant. Contact the Co-ordinating Anaesthetist on 8861 when you are free.	
3.	If you are able to attend ED then pick up one of the Resus Anaesthetist Cards from the Major Incident box in Theatre Reception and follow the actions.	
4.	You remain the On-call Anaesthetist for emergency calls so try and double with another Anaesthetist as soon as possible.	
5.	If called for a medical emergency or cardiac arrest, you are responsible for attending or finding alternative cover.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	4800	**Theatre**	
ED Majors	851033 / 4	**Enlli Ward**	4229	**2**	4822
ED Reception	851010	**Anaesthetic Dept**	4177	**3**	4823
Blood Bank	5880 / 4368	**Hospital Command Centre (HCC)**	03000 850823	**4**	4824
Haematology	#043	**Executive Seminar Room**	03000 851422	**5**	4825
Biochemistry	#045 / 4271	**CT Scan**	4692	6	4826
ITU (Cybi)	#103 / 4436 / 4244	**NWTS**	08000 848382	**7**	4827
HDU	4940	**EMRTS Air Support Desk**	0300 1232301		
		TTL Stoke or 01782 674444	07623 975903		
			Theatre Reception **Co-ordinating Anaesthetist** **ODP and Theatre Lead**	**4253** **8861 / #159** **4252**	

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YG MAJOR INCIDENT ACTION CARD:
On-call Consultant Anaesthetist

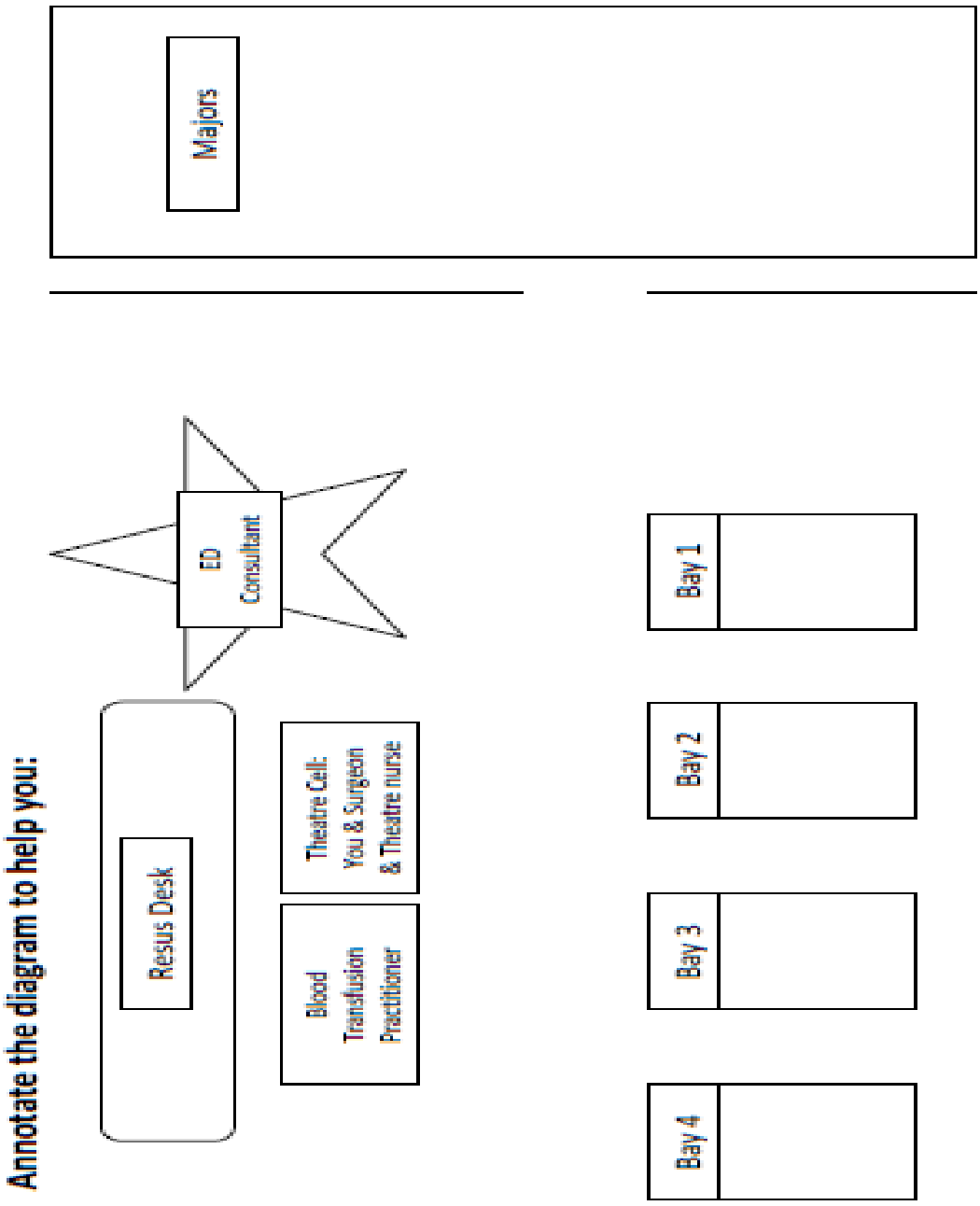
TASK	DESCRIPTION	✓
1.	Use the WhatsApp group 'YG Anaes Major Incident'. Send "MAJOR INCIDENT DECLARED. RSVP. GO TO YG THEATRE RECEPTION"	
2.	Call a second Consultant. State "Major Incident Declared". Out of hours - appoint second Consultant as admin person and tell them to follow action card for Anaesthetic Admin. Normal working hours - inform Anaesthetic Admin to follow the Anaesthetic Admin Action Card.	
3.	Make your way to the Emergency Department and meet with ED Consultant and Clinical Site Manager for briefing.	
4.	Don the 'On-call Consultant Anaesthetist' tabard.	
5.	Allocate Anaesthetist and ODP's to Resus bays and assist to make bays ready for major trauma patients. See Resus Anaesthetist Action Card.	
6.	Aim for minimum 1 ED Doc, 1 ED Nurse, 1 Anaesthetist, 1 ODP per bay. Double up Anaesthetists if possible.	
7.	Contact Co-ordinating Anaesthetist regularly to give situation reports on 8861. Give them one contact number to call you on.	
8.	Co-locate with Theatre Nurse to form Theatre Cell in Resus.	
9.	Act as the Anaesthetic Co-ordinator for ED: ALLOCATE, SUPERVISE AND SUPPORT all Anaesthetists in Resus ASSIST ED Consultant and Surgeon with TRIAGE AND DECISION MAKING (e.g. which patients to CT / Theatre first).	
10.	MAINTAIN FLOW of patients. Defer RSI and procedures until patient in theatre if safe to do so.	
11.	If moving patients to Theatre / ICU, CONFIRM DESTINATION with Co-ordinating Anaesthetist on 8861.	

12.	Contact Co-ordinating Anaesthetist on 8861 if any of the following needed: • Booking a case for theatre • Transfer Team • Mobile Medical Team • Extra staff for Resus • Cover for breaks	
13.	Document your briefs, assessments, interventions and decisions.	
14.	This is a difficult job. If you are getting fatigued, contact the Co-ordinating Anaesthetist on 8861 to arrange relief.	

NOTES:

- You are overall Lead for Anaesthetics until the Co-ordinating Anaesthetist has arrived. Then they are the Lead, but your focus will be more on supporting Triage, Resuscitation and decision making in Resus.
- All cases need to be booked by phone in Theatre Reception. Don't just send patients to Theatre. Make sure they can contact you easily.
- 551 / 063 RESPONSIBLE FOR OBS EMERGENCIES / Cardiac Arrest CALLS - Make sure they are doubled up / free to attend an emergency

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	4800	**Theatre**	
ED Majors	851033 / 4	**Enlli Ward**	4229	**2**	4822
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			Theatre Reception **Co-ordinating Anaesthetist** **ODP and Theatre Lead**	**4253** **8861 / #159** **4252**	


Majors
ED Consultant
Resus Desk
Theatre Cell: You & Surgeon & Theatre nurse
Blood Transfusion Practitioner
Bay 1
Bay 2
Bay 3
Bay 4
Annotate the diagram to help you:

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Hospital Command Centre 03000 850823

This card is for the 1st On-call Consultant Anaesthetist

YG MAJOR INCIDENT ACTION CARD:
On-call Consultant Anaesthetist

TASK	DESCRIPTION	✓
1.	Use the WhatsApp group 'YG Anaes Major Incident'. Send "MAJOR INCIDENT DECLARED. RSVP. GO TO YG THEATRE RECEPTION"	
2.	Call a second Consultant. State "Major Incident Declared". Out of hours - appoint second Consultant as admin person and tell them to follow action card for Anaesthetic Admin. Normal working hours - inform Anaesthetic Admin to follow the Anaesthetic Admin Action Card.	
3.	Make your way to the Emergency Department and meet with ED Consultant and Clinical Site Manager for briefing.	
4.	Don the 'On-call Consultant Anaesthetist' tabard.	
5.	Allocate Anaesthetist and ODP's to Resus bays and assist to make bays ready for major trauma patients. See Resus Anaesthetist Action Card.	
6.	Aim for minimum 1 ED Doc, 1 ED Nurse, 1 Anaesthetist, 1 ODP per bay. Double up Anaesthetists if possible.	
7.	Contact Co-ordinating Anaesthetist regularly to give situation reports on 8861. Give them one contact number to call you on.	
8.	Co-locate with Theatre Nurse to form Theatre Cell in Resus.	
9.	Act as the Anaesthetic Co-ordinator for ED: ALLOCATE, SUPERVISE AND SUPPORT all Anaesthetists in Resus ASSIST ED Consultant and Surgeon with TRIAGE AND DECISION MAKING (e.g. which patients to CT / Theatre first).	
10.	MAINTAIN FLOW of patients. Defer RSI and procedures until patient in theatre if safe to do so.	
11.	If moving patients to Theatre / ICU, CONFIRM DESTINATION with Co-ordinating Anaesthetist on 8861.	

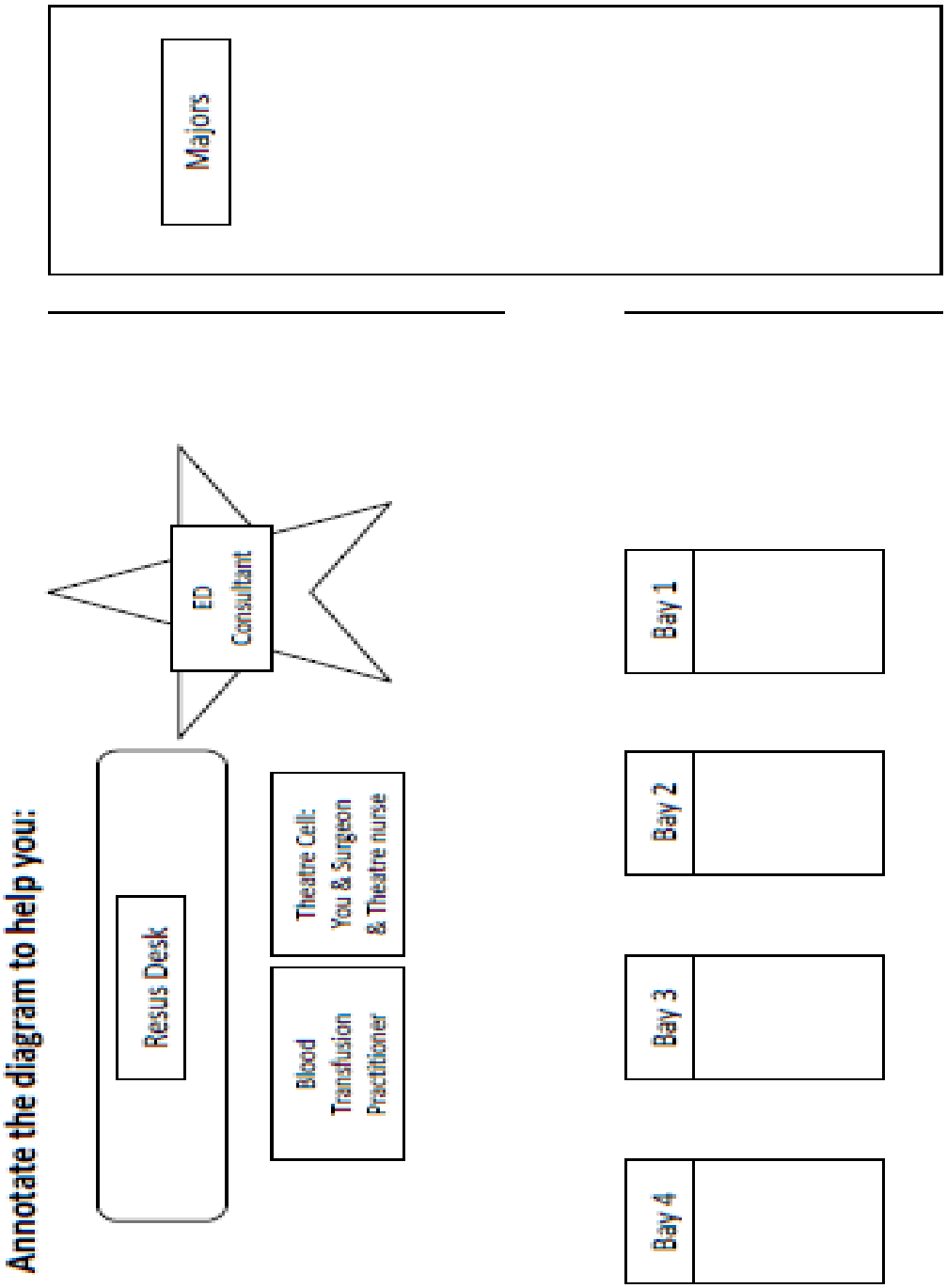
12.	Contact Co-ordinating Anaesthetist on 8861 if any of the following needed: • Booking a case for theatre • Transfer Team • Mobile Medical Team • Extra staff for Resus • Cover for breaks	
13.	Document your briefs, assessments, interventions and decisions.	
14.	This is a difficult job. If you are getting fatigued, contact the Co-ordinating Anaesthetist on 8861 to arrange relief.	

NOTES:

- You are overall Lead for Anaesthetics until the Co-ordinating Anaesthetist has arrived. Then they are the Lead, but your focus will be more on supporting Triage, Resuscitation and decision making in Resus.
- All cases need to be booked by phone in Theatre Reception. Don't just send patients to Theatre. Make sure they can contact you easily.
- 551 / 063 RESPONSIBLE FOR OBS EMERGENCIES / Cardiac Arrest CALLS - Make sure they are doubled up / free to attend an emergency

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	4800	**Theatre**	
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ITU (Cybi)	#103 / 4436 / 4244	**NWTS**	08000 848382	**7**	4827
HDU	4940	**EMRTS Air Support Desk**	0300 1232301		
		TTL Stoke or 01782 674444	07623 975903		
			Theatre Reception **Co-ordinating Anaesthetist** **ODP and Theatre Lead**	**4253** **8861 / #159** **4252**	

Hospital Command Centre 03000 850823



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This card is for the anyone appointed as Admin person by the On-call Anaesthetist
It can be: Anaesthetic Secretary, Trainee Nurse, SG or Consultant
2 people may be required initially

YG MAJOR INCIDENT ACTION CARD:
Anaesthetic Admin Person

TASK	DESCRIPTION	✓
1.	IN HOURS - Do walk round of department offices and coffee rooms and send everyone to Theatre Reception. OUT OF HOURS - Go straight to Task 2.	
2.	Call people using attached off-duty call sheet. Check the WhatsApp Major Incident Group and don't call those who have already RSVP'd. Make a note of who you called, who is coming and when. Send everyone to Theatre Reception. Once enough people have responded, go to the Anaesthetic Department.	
3.	Open Anaesthetic Department and leave it unlocked.	
4.	Pick up all bleeps and take them to Theatre Reception.	
5.	Keep Bleep 159 (Anaesthetic Admin Bleep) for yourself. Allocate 642 to the On-call Anaesthetist in ED Resus. Send a runner to take it to them.	
6.	Delegate someone to start the incident diary (kept in the Major Incident Box in Theatre Reception) and note all decisions and timings.	
7.	Help the Co-ordinating Anaesthetist in Theatre Reception: • Log where all Anaesthetists are (on the white board) • Organise breaks and send surplus people to Anaesthetic Department or home • Keep people from crowding Theatre Reception and try to keep noise levels to a minimum • Arrange next shifts including Co-ordinating Anaesthetist and yourself.	

Off Duty Callout

Use the callout sheet provided – target people near the hospital first, then people in the non-affected direction from the incident.

Hospital Command Centre 03000 850823

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	4800	**Theatre**	
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		TTL Stoke or 01782 674444	07623 975903		
			Theatre Reception **Co-ordinating Anaesthetist** **ODP and Theatre Lead**	**4253** **8861 / #159** **4252**	

This card is for the first Anaesthetic Consultant to arrive At Theatre Reception (not On-call)

YG MAJOR INCIDENT ACTION CARD:
Co-ordinating Anaesthetist

TASK	DESCRIPTION	✓
1.	If you are the first Consultant Anaesthetist at Theatre Reception, then this card is yours until you handover the position	
Stay at Theatre Reception and co-locate with Theatre and ODP Co-ordinators. Don tabard and start completing the Major Incident Whiteboard.		
2.	Contact Anaesthetic / Theatres Admin for comms and admin support. Appoint one to take calls and manage the whiteboard. If possible, have one person dedicated for each phone.	
3.	Appoint a scribe to keep the incident diary and log all comms and decisions. Consider voice recorder if available.	
4.	Contact Consultant Anaesthetist On-call for situation report (sit rep) from ED.	
5.	Contact Hospital Command Cell (HCC) on 03000 850823 and identify your position and contact number. Relay sit rep from ED.	
6.	Log in staff as they arrive and ALLOCATE ROLES (attached card) *Pair Consultants with Trainees once enough staff available.*	
7.	Contact ITU / HDU for bed status - put on Whiteboard.	
8.	Contact ITU Consultant to confirm ITU MI Plan activated.	
9.	Support the THEATRE LEADER in deciding which lists to stop and theatres to prepare.	
10.	Keep Resus Anaesthetists with their patient when they come to theatre and send a fresh (reserve) team to Resus for new patients.	
11.	Keep reserve teams in the Anaesthetic Department (Ext 4177). Make sure they are contactable. Use them to give breaks.	
12.	Send surplus staff home as soon as possible to prepare for next shift.	
13.	As cases are booked for theatre, write them on the whiteboard and allocate to a theatre in order of priority and availability.	
14.	Contact Anaesthetic Consultant in ED and ICU Consultant regularly for Sit Rep.	

15.	Contact HCC on 03000 850823 every 30 minutes to give Sit Rep.	
16.	Organise your own and admin replacement for end of shift.	

NOTES:

- The Theatre Leader is in overall charge of Theatres. Support them to make their decisions.
- The incident (from a theatre perspective) will be managed from Theatre Reception.
- Establish comms with the Anaesthetic Consultant / Theatre Cell in ED - preferably via one direct landline number.
- Consider keeping less senior staff (even novices) in elective cases where appropriate with one Consultant providing overall supervision for theatres and recovery.
- It will be busy and noisy. Try and keep people away - send them to the coffee room or Anaesthetic Department but make sure they stay contactable. Send surplus people home to prepare for the next shift.
- Ensure all patients (including normal CEPOD and trauma emergencies) are listed on whiteboard.
- Theatre Cell in ED (Consultant Anaesthetist, Surgeon and Theatre Nurse) will decide priority for theatres and will phone Theatre Reception to book bases. Actual order of cases may depend on theatre, X-ray and surgeon availability etc.
- If there is only 1 theatre free then save it for a potential time critical case that you may not yet know about.
- You need to call Resus Team (Theatre Cell) when theatre is ready to accept a patient.
- Keep Resus Anaesthetists with their patient when they come to theatre and send a fresh (reserve) team to Resus for new patients.
- 551 / 063 RESPONSIBLE FOR OBS EMERGENCIES / Cardiac Arrest CALLS - Make sure they are doubled up / free to attend an emergency.
- This is a difficult job and if you are getting fatigued, then arrange handover to another Consultant.

Hospital Command Centre 03000 850823

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	4800	**Theatre**	
ED Majors	851033 / 4	**Enlli Ward**	4229	**2**	4822
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ITU (Cybi)	#103 / 4436 / 4244	**NWTS**	08000 848382	**7**	4827
HDU	4940	**EMRTS Air Support Desk**	0300 1232301		
		TTL Stoke or 01782 674444	07623 975903		
		Theatre Reception **Co-ordinating Anaesthetist** **ODP and Theatre Lead**	**4253** **8861 / #159** **4252**		

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Role	Name	Time
Immediate Roles – Allocate First		
Consultant On-call in Resus	(1st on call Consultant)	
Resus 1		
Resus 2		
Resus 3		
Resus 4		
ICU Resident 1	(the on-call ICU resident)	
Theatre Anaesthetists	**Keep the Resus Anaesthetics with their patient when they come to theatre and send a fresh (reserve) team to Resus.** **Consider keeping less senior staff in elective cases with one Consultant providing overall supervision for theatres and recovery.**	
Theatre / Recovery Consultant		
ICU Consultant (if not already on rota)	Choose an ICU Consultant who does regular ICU sessions)	
Reserve team 1 (e.g. for Resus)		
2nd ICU Consultant (to run auxiliary ICU)		
ICU Resident 2		
Reserve Team 2		
ED Majors Team		
Recovery / ICU Resident 3		
Reserve Team 3		
Secondary Roles		
Mobile Medical Team	Choose people with pre-hospital experience if possible	
Transfer Team 1		
Transfer Team 2		
Replacements for On-call Team		
Replacement for Co-ordinating Anaesthetist		
Staff for Next Shift		

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YG MAJOR INCIDENT ACTION CARD:
Obstetric On-call Anaesthetist

TASK	DESCRIPTION	✓
1.	Your priorities are Obstetrics. You must remain free to attend obstetric emergencies.	
2.	If not busy on Obstetrics, contact Co-ordinating Anaesthetist (Theatre Reception / 8861) or if they are not available, contact the On-call Consultant Anaesthetist in ED.	
3.	If they are busy, go to ED Resus and help prepare for major trauma patients (see Resus Anaesthetist Action Card).	
4.	Ensure you are doubled up asap and free to attend obstetric emergencies.	
5.	If not required in Resus, go to ITU and help review patients suitable for discharge and prepare for new admissions.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	4800	**Theatre**	
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ITU (Cybi)	#103 / 4436 / 4244	**NWTS**	08000 848382	**7**	4827
HDU	4940	**EMRTS Air Support Desk**	0300 1232301		
		TTL Stoke or 01782 674444	07623 975903		
			Theatre Reception **Co-ordinating Anaesthetist** **ODP and Theatre Lead**	**4253** **8861 / #159** **4252**	

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This card is for all Anaesthetists on a theatre list (inc Theatre 2 Trauma list)

YG MAJOR INCIDENT ACTION CARD:
All Anaesthetists on a Theatre List

TASK	DESCRIPTION	✓
1.	If you are doubled up, then if appropriate, the more junior person should stay and the more senior person should contact the Co-ordinating Anaesthetist in Theatre Reception / 8861.	
2.	Contact Theatre Lead (Theatre Reception / 4253) when safe to do so.	
3.	Finish case you are on and do not send for any more unless Major Incident Stand Down called or Theatre Lead allows list to continue.	
4.	Discuss with Surgeon which cases can be cancelled or postponed for next 24 hours and which need to be done today.	
5.	Report to Theatre Reception if your list is finished.	
6.	Help to prepare theatres to receive patients.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	4800	**Theatre**	
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			Theatre Reception **Co-ordinating Anaesthetist** **ODP and Theatre Lead**	**4253** **8861 / #159** **4252**	

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This card is for all Anaesthetists who attend ED Resus. Only go to ED Resus if your Action Card or the Co-ordinating Anaesthetist tell you. Otherwise go to Theatre Reception

YG MAJOR INCIDENT ACTION CARD:
Resus Anaesthetists (8 Cards)

TASK	DESCRIPTION	✓
1.	Go to ED Resus, report to On-call Consultant Anaesthetist and prepare bays for major trauma patients.	
2.	Prepare drugs for RSI, Analgesia, TXA, Antibiotics, Emergency drugs. You may need to take drugs from Theatres.	
3.	Check equipment: Airway, Ventilator, IC/IO Access, Monitors, Syringe Drivers.	
4.	Aim for at least 1 Anaesthetist, 1 ODP, 1 ED Dr and 1 Nurse per bay. If possible double up - 1 Consultant with 1 Trainee.	
5.	Be prepared to act as Team leader for your bay if required.	
6.	Prepare blood bottles and forms for X-match, ROTEM, ABG, FBC.	
7.	Use hospital D Number, DoB and name on all forms for blood request, CT, theatres etc. Unknown patients will be given an allocated name and estimated DoB on booking in.	
8.	Prepare for the unexpected. Pre-alert information may be scanty, absent or wrong.	
9.	Assist with all critical care interventions, access, analgesia.	
10.	Speak to On-call Consultant Anaesthetist in Resus if you need support or advice.	
11.	Stay with your patient unless you are retasked.	
12.	If patient needs blood: liaise with Blood Transfusion Practitioner in ED (or call lab on 5880 if they are not there). Use sparingly and use O+ve for males.	
13.	If patient needs RSI: inform On-call Consultant Anaesthetist in Resus. If patient going to theatre, consider deferring RSI until in theatre. Always use the RSI checklist.	

Hospital Command Centre 03000 850823

14	If patient needs surgery, inform the Theatres Cell / Consultant Anaesthetist in ED Resus (or call Theatre Reception on 4252).	
15.	Don't take patient to theatre until theatres have confirmed they are ready.	
16.	Stay with <u>your</u> patient if they go to theatre.	
17.	Ensure you have signed in and document you assessment, interventions and decisions clearly in the patient's notes.	
18.	If patient needs transfer to another hospital, contact the Co-ordinating Anaesthetist on 8861.	

COMMUNICATIONS					
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		Theatre Reception	**4253**		
		Co-ordinating Anaesthetist	**8861 / #159**		
		ODP and Theatre Lead	**4252**		

This card is for the Consultant Intensivist

YG MAJOR INCIDENT ACTION CARD:
On-call Intensive Care Consultant

TASK	DESCRIPTION	✓
1.	Report to Theatre Reception with ICU Nursing Co-ordinator for METHANE Report from Co-ordinating Anaesthetist (8861).	
2.	Review critical care bed state with ITU Nursing Co-ordinator.	
3.	Decide with Co-ordinating Anaesthetist (8861): • Need to escalate ITU capacity • ITU Staffing needs • Communicate with Hospital Incident Commander 03000 840490 regarding transfer of existing Enlli patients.	
4.	Decide on movements of existing and new Level 3 patients across ITU/HDU (benefit of cohorting new patients on ITU vs time/personnel loss in movements and overwhelming one location with new patients). • Identify critical care DTOC's for discharge to ward • Identify critical care patients for outlying to Enlli (must be Level 2 patients or DTOC's)	
5.	Allocate available staff to clinical areas: • Ensure ITU, HDU and Enlli each have Consultant and Resident Anaesthetist • Allocate MET call / cardiac arrest response to one team • Confirm nursing needs with ICU Nursing Co-ordinator	
6.	Ongoing role: • Once Ward teams allocated, do not become involved in direct patient care where possible • Accept and discharge incident elated and non-incident patients, informing Co-ordinating Anaesthetist • If inadequate critical care capacity develops, inform Co-ordinating Anaesthetist (8861) • Support critical care ward teams equipment and skills requirements • Hourly sit-reps to Co-ordinating Anaesthetist (8861) and Major Incident Commander • Check with Co-ordinating Anaesthetist for ongoing staffing needs on future shifts	
7.	Discuss with Co-ordinating Anaesthetist regarding bed de-escalation.	

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Hospital Command Centre 03000 850823

This card is for Anaesthetists (Consultants, Junior Teams) covering or allocated to clinical care

YG MAJOR INCIDENT ACTION CARD:
Critical Care Doctor Teams

TASK	DESCRIPTION	✓
1.	Ensure your team has a Consultant and Junior Anaesthetist / Intensivist.	
2.	Once allocated to role, report to Seminar Room for tasking from ICU On-call Consultant and ITU Nursing Co-ordinator who will allocate a critical care ward (Cybi, HDU, Enlli).	
3.	Communicate with ICU On-Call Consultant and Nursing Co-ordinator and ward Nurse in Charge only.	
4.	Do not leave your ward without informing the Consultant On-call.	
5.	Roles: • Ensure adequate equipment / monitoring and drugs available in your area • Communicate shortages with ICU Nursing Co-ordinator • Ensure daily ward review done on existing patients if able • Receive handover from transferring ED / Theatre Teams • Ensure secondary and tertiary survey are completed • Arrange investigations and ongoing care of new and existing patients • Alert ITU Consultant On-call of all patients ready for discharge • Respond to arrest calls only if role allocated to you	
6.	Alert ITU Consultant On-call if extra resource or staffing relief required	

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YG MAJOR INCIDENT ACTION CARD:
ITU Nursing Co-ordinator

TASK	DESCRIPTION	✓
1.	Allocate a senior member of staff to assume role of Nurse in Charge of existing ITU/HDU patients.	
2.	Base yourself in the Seminar Room on Cybi Ward - you will remain supernumerary throughout the incident.	
3.	Report to the Co-ordinating Anaesthetist (Theatre Reception) and inform them of bed / staff status and obtain details of the incident.	
4.	Instruct a member of your team to call in sufficient staff, using dedicated 'WhatsApp' group for the immediate situation and place others on standby.	
5.	Instruct a member of your team to call in sufficient staff, using dedicated 'WhatsApp' group, for the immediate situation and place others on standby.	
6.	Liaise with the ITU Resident or Consultant if available, of patients that may be safely transferred out of the unit.	
7.	Arrange for the transfer of these patients **liaising with Bed Managers.**	
8.	Allocate one staff member to act as a runner between ED and ITU, instruct the nurse to report back to you at regular intervals.	
9.	Ascertain bed status in other North Wales and North West ITU's.	
10.	Discuss with Co-ordinating Anaesthetist (Theatre Reception), Hospital Incident Commander (03000 840490), ITU Anaesthetist about whether to activate the Auxiliary ITU Plan.	
11.	Ensure that arrangements are made for rest / meal breaks and relief for your staff.	
12.	Co-ordinate shift change as appropriate.	
13.	Ensure all staff report to you before leaving the hospital to be debriefed and thanked.	

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This card is for Nurse in Charge of each Critical Care Ward Area

YG MAJOR INCIDENT ACTION CARD:
Critical Care Ward Nurse in Charge

TASK	DESCRIPTION	✓
1.	Ensure Consultant in Charge of your ward knows you are the Nurse in Charge.	
2.	Obtain Bleep numbers from your Medical Team.	
3.	Ensure adequate equipment / ventilator / monitoring / drugs / documentation for each bed space.	
4.	Create a status board for your area.	
5.	Contact ITU Nursing Co-ordinator with hourly updates.	
6.	Ensure ad3equate rest periods.	
7.	Ensure all staff report to ITU Nursing Co-ordinator prior to leaving the hospital.	

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YG MAJOR INCIDENT ACTION CARD:
Critical Care Nurse

TASK	DESCRIPTION	✓
1.	Report to the Nursing Co-ordinator in the Seminar Room on Cybi.	
2.	Be directed by the Nursing Co-ordinator, do not undertake any tasks which have not been allocated to you by the Co-ordinating Nurse.	
3.	Once allocated to specific critical care ward, do not undertake any tasks not allocated to you by your Ward Nurse in Charge.	
4.	If allocated to Enlli - obtain list of items needed from Major Incident Box. Refer to Enlli as 'Enlli Ward' at all times - not 'HDU'.	
5.	If you need additional support or rest at any time, inform Ward Nurse in Charge.	
6.	If allocated to another area of the hospital i.e. ED, report to the Nursing Co-ordinator on ITU at regular intervals. Do not leave the hospital without the knowledge of the Co-ordinating Nurse.	
7.	Wait in the coffee room on Cybi if you are not needed immediately.	
8.	Inform the Nursing Co-ordinator before you leave the hospital so you can be debriefed and thanked.	

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YG MAJOR INCIDENT ACTION CARD:
ITU Ward Clerk

TASK	DESCRIPTION	✓
1.	Report to ITU Nursing Co-ordinator in the ITU Seminar Room.	
2.	Do not leave critical care without informing the ITU Nursing Co-ordinator.	
3.	Refer all calls regarding bed states or incident management to ITU Nursing Co-ordinator.	
4.	Ongoing Roles: • Update PiMS for all admissions, discharges and transfers • Ensure notes are in order for all new admissions and discharges • Contact Ward Nurse in Charge for all patient enquiries	
5.	Report to ITU Nursing Co-ordinator prior to leaving the hospital.	

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This card is for the Person in Charge of Theatres (Bleep 019/611)

YG MAJOR INCIDENT ACTION CARD:
Person in Charge of Theatres

TASK	DESCRIPTION	✓
1.	You will be the Co-ordinator during major incident until you delegate to a more senior colleague on their arrival.	
2.	Theatre Department will be contacted by Switchboard to declare 'Major Incident'.	
3.	Remain in Theatre and base yourself in the Main Theatre Reception Ext 4252/4253.	
4.	Inform a Theatre Nurse/Recovery or ODP to follow the 'Action Card Theatre Co-ordinator in ED'.	
5.	Inform Co-ordinating Anaesthetist (Ext 8861) that you are based in Theatre Reception	
6.	Phone 1st available Team Leader and instruct them to begin contacting non-on duty staff at home to request assistance – starting with Team Leaders. (A copy of contact numbers is to be kept in recovery in a sealed envelope).	
7.	Hand out Action Cards for the runners.	
8.	Hand out Action Cards for the Telephone Operators.	
9.	Hand out Action Card for the Scribe/Loggist.	
10.	Initiate preparation of 2 x General and 2 x Trauma theatres for cases until further instructions are given.	
11.	Contact CSSD to inform them of impending need.	
12.	Ensure that the 'on duty' ODO is aware of the incident.	
13.	Ensure that relief breaks are organised for all scrub/circulating or recovery staff.	
14.	If staff is not required then send them home as soon as possible to ensure there are rested staff for further shifts or later routine work.	
15.	**It is important to carry hospital ID badges at all times during the incident.**	

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YG MAJOR INCIDENT ACTION CARD:
Theatre Co-ordinator in ED

TASK	DESCRIPTION	✓
1.	The ODP must proceed to Main Reception and will be delegated as the Co-ordinator between Theatres and ED during a Major Incident.	
2.	Co-locate yourself in ED **Use the appropriate tabard** to communicate between Theatres and ED.	
3.	You will liaise with the Co-ordinating ODP/Anaesthetist/ Person in Charge .	
4.	You are in charge of informing ED that the theatre department is ready to take patient.	
5.	You will inform theatre that the ED patient is on the way to theatre.	
6.	You will liaise with the person in charge in theatre if there is any change in requirements in your area of if you need relief.	
7.	You will report to the Co-ordinating ODP before you leave the hospital to be thanked and debriefed.	
8.	**It is important that you carry your hospital ID badges and bleeps throughout the incident.**	

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YG MAJOR INCIDENT ACTION CARD:
Co-ordinating ODP

TASK	DESCRIPTION	✓
1.	This will be the first Senior non on-call ODP to arrive in theatre. The Co-ordinating ODP will be based in Theatre Reception Ext 4252/4253 and will deploy ODP staff during the incident. The Co-ordinating ODP will be supernumerary whilst in this role.	
2.	They will report to the Co-ordinating Anaesthetist Ext 8861 on arrival in the department. If there is none present, report to the Hospital Incident Commander Ext 840490 and select this Action Card from the Major Incident file.	
3.	Ensure that the on Duty ODO is contacting other members of his/her team at home.	
4.	Liaise with the Co-ordinating anaesthetist Ext 8861 to ensure your staff are deployed to maximise efficiency depending on case mix and load expected in theatres. Remember that there may be trauma and obstetrics cases and those routine emergencies still need to be dealt with.	
5.	Liaise with the bleep 611/019 (person in charge of theatres)	
6.	**If the incident happens during working hours try to ascertain estimated finish times of lists and relay this information to the Co-ordinating Anaesthetist**	
7.	Ensure that arrangements are made for rest/meal breaks and relief to the ODPs working.	
8.	Initially ensure that extra ODPs remain in the department until you have ascertained the level of support that is likely to be required.	
9.	When the extent of the incident has become clear those not needed immediately should be sent home as soon as possible – they will be the 'next shift'	
10.	Ensure all staff report to you before leaving, to be debriefed and thanked.	
11.	Don't forget relief will be needed for you.	

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YG MAJOR INCIDENT ACTION CARD:
Lead Operating Department Practitioner (ODP)

TASK	DESCRIPTION	✓
1.	The lead ODP needs to task one ODP to liaise with the ITU Consultant to assist in setting any equipment needed in other areas.	
2.	**If needed:**	
3.	The ODP must proceed to main reception and pick up master key to unlock the doors to the Day Surgical Unit. If it is locked (side door c4579y). During the day all operating list will be stopped and the Day Surgery Unit cleared by Day Surgery staff.	
4.	The Lead ODP must organise the removal of ventilators and monitors from the anaesthetic rooms	
5.	Non on-call staff will proceed to theatres via the main entrance if requested. This first senior to arrive will become the Co-ordinating ODP and remain supernumerary through the incident. They will be based in Theatre Reception Ext 4252/4253. They will liaise with the Co-ordinating Anaesthetist Ext 8861.	
6.	All ODP's will report to the Co-ordinating ODP: • On arrival • If in need of relief • If required to move out of their area • Before leaving the hospital	

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YG MAJOR INCIDENT ACTION CARD:
Operating Department Practitioners (ODP)

TASK	DESCRIPTION	✓
1.	The obstetric bleep holder (510) will be contacted by Switchboard informing them of **'Major Incident Standby – please be prepared to assist OR Major Incident Declared – please report to theatres'**	
2.	If the call is **Standby –** The Obstetric bleep holder must contact the 1st on call ODP.	
3.	Contact the 2nd on call ODP and inform them of the standby. Stay where you are. You will get a further call from switch.	
4.	If the call is **declared** – Obstetrics ODP will contact the 2nd on call ODP informing them of declaration.	
5.	Following declaration the 2nd on call will contact his/her colleagues starting with the seniors. Once contact has been made, that person will be delegated to contact all other ODPs stating that a Major Incident has been declared and that assistance is required – please proceed to theatres. *(Tick box against all phone numbers to show availability.)*	
6.	The 1st on call ODP will report to (or contact) the Co-ordinating Anaesthetist who will be situated in Theatre Reception Ext 4252/4253, or if not yet arrived the Hospital Incident Commander Ext 840490.	
7.	The 1st on call ODP will then proceed to theatres to open and start preparing a theatre to accept a major general surgical emergency.	
8.	The first Senior non on-call ODP to arrive will become the Lead ODP	
9.	It is the responsibility of the ODP Department to ensure that staff contact numbers are available at all times, including at home.	
10.	The 2nd on call must report to the co-ordinating ODP in Theatre Reception Ext 4252/4253 following assurance that other ODPs are being contacted.	
11.	The Obstetrics ODP will be contacted by Switchboard via the bleep system with the Phase 2 staff when the major incident has been declared. They will proceed to Theatres and report to the Lead ODP if available and begin to prepare a theatre. **(Remember that you must be available for obstetric theatre.)**	

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This card is for the Theatres Telephone Operator

YG MAJOR INCIDENT ACTION CARD:
Theatres Telephone Operator

TASK	DESCRIPTION	✓
1.	The Theatres Telephone Operator must allocate themselves in Theatre Reception.	
2.	Make sure you have enough of the Telephone Operator sheets and working pens.	
3.	Write down all information received over the telephone.	
4.	Report every phone call back to the person in charge Bleep 611/019 or the Lead Anaesthetist.	
5.	Report to the person in charge if: • In need of relief • If required to move out of the area • Before leaving the hospital	

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YG MAJOR INCIDENT ACTION CARD:
Operating Department Orderly (ODO)

TASK	DESCRIPTION	✓
1.	You will be alerted by the Nurse in Charge of Theatres that a major incident has been declared.	
2.	You will locate the list of ODOs and their home telephone numbers and begin calling them in to the hospital starting with the more senior staff member.	
3.	You will assist the nursing staff in opening and beginning to prepare theatres.	
4.	You will report to the Co-ordinating ODP in Theatre Reception, to inform them who you have managed to contact and who is coming in.	
5.	You will then be directed by the Co-ordinating ODP as to where you should go.	
6.	Always inform the Co-ordinating ODP when your job is complete or if you need additional support or rest.	
7.	Return to the ODO room if you are not working.	
8.	Report to the Co-ordinating ODP before you leave the hospital to be debriefed and thanked.	

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This card is for the Theatre staff not on duty

YG MAJOR INCIDENT ACTION CARD:
Theatre Staff NOT ON DUTY

TASK	DESCRIPTION	✓
1.	You will be contacted by a member of theatre staff stating a Major Incident has been declared and requesting that you attend the hospital.	
2.	You will: • Proceed to theatres, entering the hospital by the Main Entrance • You will report to Bleep 611/019 who will be in Theatre Reception • You will go to the specified area and prepare as directed	
3.	Follow the Action Card for Runners for rest of the instructions.	

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YG MAJOR INCIDENT ACTION CARD:
Theatre Runners

TASK	DESCRIPTION	✓
1.	You will be informed by the Bleep 611/019 holder that a major incident has been declared.	
2.	Start preparing theatres as directed by the Bleep 611/019.	
3.	You must inform the Person in Charge of Theatre of any additional need that you may have in the area in which you are working, or if staff requires rest.	
4.	You must inform the Person in Charge of Theatre if you need to leave the area in which you have been placed.	
5.	If you are not allocated to an area you should wait in the coffee room so you can be located quickly if needed.	
6.	You must inform the Person in Charge of Theatre before you leave the hospital so you can be debriefed and thanked.	
7.	**Remember to wear your hospital ID badge at all times.**	

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